June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

RE: CMS-0042-NC, Request for Information; Health Technology Ecosystem.

Submitted electronically on regulations.gov

Dear Administrator Oz:

On behalf of the Massachusetts Telemedicine Coalition (*t*MED), representing more than 50 healthcare provider organizations, consumer advocates, technology organizations, and telecommunication associations in Massachusetts, we appreciate the opportunity to submit comments to the Centers for Medicare & Medicaid Services (CMS) on the Request for Information; Health Technology, published in the *Federal Register* on May 16, 2025. Our comments highlight key recommendations as well as areas of concern, focusing on telehealth, in particular.

The Massachusetts Experience

Based on the March 2024 report from the Massachusetts Center for Health Information and Analysis,[1] in 2022, telehealth spending reported by commercial, Medicare Advantage, and MassHealth plans totaled $1.6 *billion* (2.4% of total healthcare spending in the state). Overall, telehealth spending in 2022 remained significantly higher than the $3 *million* reported in the pre-pandemic year 2019 in Massachusetts. However, it has also become clear that telehealth has expanded access to services to patients, particularly for behavioral health services.

Spending for telehealth services delivered by "other professionals" (non-physician providers), which includes occupational and physical therapists, nurse practitioners, physician assistants, and

[1] *Massachusetts Center for Health Information & Analysis, "Performance of the Massachusetts Health Care System: March 2024 Annual Report", March 2024, https://www.chiamass.gov/assets/2024-annual-report/2024-Annual-Report.pdf*

certain behavioral health providers, represented about half (53.8%) of all telehealth spending in 2022, consistent with prior years.

In particular, it is notable that 10 diagnosis categories accounted for more than two-thirds of telehealth spending for commercially insured Massachusetts residents and, of these diagnosis categories, nine of the top 10 diagnosis categories were behavioral health conditions, consistent with trends in the types of providers who are providing a large portion of their services via telehealth.

Congressional Action is Needed

The *t*MED Coalition fully supports immediate action from Congress to make important telehealth flexibilities permanent prior to the statutory expiration of those flexibilities on September 30, 2025. Acting as soon as possible will provide much needed certainty and safeguard this important policy.

Clinicians need telehealth to expand access to care and support strong patient relationships, and they value the flexibility created by the option for remote care when clinically appropriate. Safety net providers, such as community health centers and rural health clinics, have depended on these flexibilities. Current telehealth flexibilities have played a critical role in promoting access to vital healthcare services, including those provided by advanced specialists (such as oncologists) and mental health practitioners, without requiring a previous in-person appointment. This access benefits are particularly critical for patients in rural and underserved areas, those with mobility issues, and patients with transportation or other limitations that prevent them from accessing in-person care in a timely manner.

Predictability in telehealth policy now, in advance of the September 30 deadline, would:

- Create certainty for Medicare beneficiaries, who will otherwise face confusion as to whether they will have continued access to clinicians and services they are using virtually.

- Strengthen our national healthcare workforce by enabling a greater number of clinicians to provide telehealth services and allowing for investment in flexible virtual staffing models that address current workforce shortages while maintaining high-quality healthcare.

- Ensure continued investment in the technology tools and infrastructure needed to offer telehealth services, particularly for smaller providers serving the rural and underserved, who cannot afford to invest in these tools without a reimbursement pathway.

Without clear guidance from Congress on a long-term approach for telehealth flexibilities, healthcare delivery systems will not be able to invest sufficient resources to allow maximum access to patients. We believe that policymakers now have more than enough evidence to see the benefits of telehealth so that they help create a permanent pathway to ensure that telehealth continues to be available and accessible.

We appreciate CMS' focus on this critical issue, and we invite you to join us in urging Congress to make the following specific telehealth flexibilities permanent before they expire on September 30, 2025:

- Removing geographic restrictions and expanding originating sites to include any site at which the patient is located, including the patient's home.

- Extending the Acute Hospital at Home waiver so that hospital may continue to take advantage of this innovative program for patients in their residences.

- Expanding the ability for eligible practitioners to furnish telehealth services to include occupational therapists, physical therapists, speech-language pathologists, and audiologists.

- Extending the ability for federally qualified health centers (FQHCs) and rural health clinics (RHCs) to furnish telehealth services, and allowing critical access hospitals (CAHs) to have the same ability to offer and bill for telehealth services by including language to include CAHs as eligible distant sites.

- Removing the six-month in-person requirement for mental health services furnished through telehealth, including the in-person requirements for FQHCs and RHCs.

- Extending coverage and payment for audio-only telehealth services.

- Extending the ability to use telehealth services to meet the face-to-face recertification requirement for hospice care.

Additionally, the *t*MED Coalition encourages CMS to reinstate remote virtual services for Partial Hospital Programs (PHP). Many of our member hospitals offer PHP services, and during the Public Health Emergency implemented during the first Trump administration, patients benefitted enormously through Medicare's coverage of virtual PHP services. Unfortunately, Medicare members are not currently afforded the same virtual treatment options as non-Medicare members, unless they are able to pay out of pocket.

We urge CMS to reinstate virtual PHP services for Medicare beneficiaries since these services:

- achieve similar clinically equivalent outcomes, such as reduction in symptoms, improved functioning, and participant satisfaction as in-person care;

- improve attendance and adherence to treatment and reduce barriers such as transportation, caregiving responsibilities, disability barriers for wheelchair bound patients, and access despite geographic distance or rural isolation;

- sustain treatment for those who suffer from disabilities and chronic medical conditions that make in-person attendance extremely difficult;

- allow those who may have a contagious illnesses to participate in virtual PHP without jeopardizing others;

- support workforce retention for staff who are at risk of leaving the field due to health concerns, caregiving responsibilities, or geographic constraints; and

- maintain continuity of care for discharged patients following a psychiatric hospitalization, thereby helping to reduce more costly inpatient re-admissions.

Thank you for your consideration of these comments. Collectively, the *t*MED Coalition strongly urges the adoption of the policies as outlined above that will protect and continue to provide opportunities for healthcare via telemedicine for our patients. Should you have any questions or concerns, please contact Adam Delmolino, senior director, virtual care & clinical affairs, at the Massachusetts Health & Hospital Association at adelmolino@mhalink.org.

Sincerely,

*The members of t*MED - The Massachusetts Telemedicine Coalition

- Massachusetts Health & Hospital Association
- Massachusetts Medical Society
- Massachusetts League of Community Health Centers
- Conference of Boston Teaching Hospitals
- Massachusetts Council of Community Hospitals
- Hospice & Palliative Care Federation of Massachusetts
- American College of Physicians – Massachusetts Chapter
- Highland Healthcare Associates IPA
- Health Care For All
- Organization of Nurse Leaders
- HealthPoint Plus Foundation
- Massachusetts Association of Behavioral Health Systems
- Massachusetts Academy of Family Physicians
- Seven Hills Foundation & Affiliates
- Case Management Society of New England
- Massachusetts Association for Occupational Therapy
- Atrius Health
- New England Cable & Telecommunications Association
- Association for Behavioral Healthcare
- National Association of Social Workers – Massachusetts Chapter
- Massachusetts Psychiatric Society
- Massachusetts Early Intervention Consortium
- Digital Diagnostics
- American College of Cardiology – Massachusetts Chapter
- The ALS Association

- Zipnosis
- Perspectives Health Services
- Bayada Pediatrics
- Planned Parenthood League of Massachusetts
- Mass. Family Planning Association
- BL Healthcare
- Phillips
- Maven Project
- Upstream USA
- Cambridge Health Alliance
- Heywood Healthcare
- Franciscan Children's Hospital
- American Physical Therapy Association – Massachusetts
- Community Care Cooperative
- Fertility Within Reach
- Virtudent
- Resolve New England
- Massachusetts Association of Mental Health
- AMD Global Telemedicine
- hims | hers
- Asian Women for Health
- Massachusetts Society of Clinical Oncologists
- Reproductive Equity Now
- Recovery Centers of America
- Massachusetts Chapter, American Academy of Pediatrics
- Massachusetts Speech and Hearing Association
- Southcoast Health
- Massachusetts Orthopedic Association
- Transhealth
- Massachusetts Academy of Nutrition & Dietetics